



0000016518564

Name:	Pooja	Age/Gender:	47 Year(s) 0 Month(s) 0 Day(s)/Female
Referred By:	self	Client Name:	N.A
Collection Date:	13-07-2026 12:35:52	Report Release Date:	14-07-2026 15:29:08

No.	Investigation	Observed Value	Unit	Biological Reference Interval
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Widal Test (Tube Agglutination)

1	Salmonella Typhi - 'O' Antigen Serum, Method: Tube Agglutination	No agglutination		No agglutination
2	Salmonella Typhi - 'H' Antigen Serum, Method: Tube Agglutination	No agglutination		No agglutination
3	Salmonella Paratyphi - A - H Antigen Serum, Method: Tube Agglutination	No agglutination		No agglutination
4	Salmonella Paratyphi B - H Antigen Serum, Method: Tube Agglutination	No agglutination		No agglutination

O antigen Agglutination titre	H antigen Agglutination titre	Interpretation
No agglutination	Low titres <160	Anamnestic reaction/cross reacting antibodies.
Low titres <80	No agglutination	Anamnestic reaction/cross reacting antibodies.
Low titres <80	Low titres <160	Confirm rise in titres with repeat specimen after 2-3 weeks.
>= 80	No agglutination	Suggestive of Enteric fever.
No agglutination	>= 160, any one of either SH, STA or STB antigen.	Suggestive of Enteric fever
>= 160	>= 160, any one of either SH, STA or STB antigen.	Strongly indicate Enteric fever.
Agglutination present(any titre)	Agglutination (any titre), more than one of SH, STA or STB antigen types.	Seen generally post immunization.

Interpretation

- For O antigen, titres of 80 or above can be significant.
- For H antigen, titres of 160 or above are considered significant.
- Demonstration of rising titres in paired sera is confirmatory.
- SH=S.typhi, STA=S.paratyphi A, STB=S.paratyphi B.



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End Of Report

Authorized Signatory
Dr. N. Das
MBBS,MD (Pathology)
Consultant Pathologist.



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Collection Date:	13-07-2026 12:35:25	Report Release Date:	13-07-2026 18:58:22

No.	Investigation	Observed Value	Unit	Biological Ref. Interval
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Complete Haemogram Test

Erythrocytes

1	Total Red Blood Cell Count (RBC)	4.6	10 ⁶ /μL	4.1-5.1
2	Hemoglobin	12.9	g/dL	12.3-15.3
3	Hematocrit (PCV)	39.9	%	33-57
4	Mean Corpuscular Volume (MCV)	85.8	fL	80-96
5	Mean Corpuscular Hemoglobin (MCH)	27.7	pg	27.5-33.2
6	Mean Corpuscular Hemoglobin Concentration (MCHC)	32.3	g/dL	29.4- 34.5
7	Red Cell Distribution Width (RDW-CV)	15.5	%	12-15
8	Red Cell Distribution Width-SD(RDW-SD)	48.3	fL	32-60.4
9	Nucleated Red Blood Cells	0.0	cells/μL	0 - 1.36
10	Nucleated Red Blood Cells Percentage	0.0	%	0-4

Platelets

11	Platelet Count	268.0	10 ³ /μL	150-450
12	Mean Platelet Volume (MPV)	10.0	fL	6 - 12
13	Platelet Distribution Width (PDW)	15.4	%	15.5-18.3
14	Plateletcrit (PCT)	0.269	%	0.12-0.37

Leucocytes

15	Total Leucocytes Count	7.2	10 ³ /μL	4.4-11
16	Neutrophils	58.2	%	40-77
17	Lymphocyte Percentage	33.6	%	16-44
18	Monocytes Percentage	5.4	%	2.0-10.0
19	Eosinophils Percentage	2.6	%	0-7
20	Basophils Percentage	0.2	%	0 - 1
21	Neutrophils-Absolute Count	4.19	10 ³ /μL	1.8-7.8
22	Lymphocytes-Absolute Count	2.42	10 ³ /μL	1-4.8
23	Monocytes-Absolute Count	0.39	10 ³ /μL	0.1-1.0
24	Eosinophils-Absolute Count	0.19	10 ³ /μL	0 - 0.45



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Leucocytes

25	Basophils-Absolute Count	0.01	10 ³ /μL	0-0.2
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Peripheral Blood Smear

26	RBC Morphology	Normocytic Normochromic
27	WBC Morphology	Within Normal Range
28	Platelets	Adequate On Smear

Interpretation

Sample type: EDTA whole blood.

Test Methods:

RBC/WBC/Platelets: Impedance method,

Hemoglobin: Photometric measurement,

Differential count: VCSn Technology,

MCV, MPV: Measured parameter Indices,

Absolute counts: Calculated.

(Processed on Fully Automated 5 parts differential Hematology analyzer).



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Thyroid Profile - Total T3, Total T4, TSH (TFT)

1	Total T3 Serum, Method: CLIA	104.73	ng/dL	60 - 200
2	Total T4 Serum, Method: CLIA	9.43	µg/dL	4.5 - 14.5
3	TSH (Thyroid Stimulating Hormone) Serum, Method: CLIA	2.963	µIU/ml	0.35 - 5.5

Interpretation

- Triiodothyronine (T3) is produced by the thyroid gland and along with thyroxine (T4) help control the rate at which the body uses energy. Elevated T3 denote hyperthyroidism while low levels indicate hypothyroidism.
- The most common causes of thyroid dysfunction are related to autoimmune disorders. Graves disease causes hyperthyroidism, but it can also be caused by thyroiditis, thyroid cancer, and excessive production of TSH. Total T3 is used to assess thyroid function.
- Elevated T4 levels may indicate hyperthyroidism. They may also indicate other thyroid problems, such as thyroiditis or toxic multinodular goiter. Abnormally low levels of T4 may indicate: dietary issues, such as fasting, malnutrition, or an iodine deficiency, medications that affect protein levels, hypothyroidism, illness.
- Thyroid-stimulating hormone (TSH) stimulates the production and release of T4 (primarily) and T3. They help control the rate at which the body uses energy and are regulated by a feedback system. Most of the T4 circulates in the blood bound to protein, while a small percentage is free (not bound).
- Lab has estimated Total T4 reference intervals that are specific for India, using the indirect sampling technique following CLSI EP28-A3c document: Defining Establishing, and Verifying Reference Intervals in the Clinical Laboratory: Approved Guideline-Third Edition.
- Thyroid hormone status during pregnancy:

Pregnancy stage	TSH (µIU/ml)	T3 (ng/dl)	T4 (µg/dL)
First trimester	0.05-3.70	71-175	6.5-10.1
Second trimester	0.31-4.35	91-195	7.5-10.3
Third trimester	0.41-5.18	104-182	6.3-9.7

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QUALITY POLICY

GENERAL DIAGNOSTICS INTERNATIONAL (P) Ltd. maintains the highest standards of quality control in all aspects of laboratory work. The purpose of our laboratory's Quality Management System is to ensure that:

- Principles of all accreditations, including that of NABL – ISO1518:2012 (National Accreditation Board of Laboratories) are adhered for each test in the scope of the accreditation, and beyond.
- Test methods, processes and control mechanisms are timely updated and fully validated to ensure the accuracy and reliability of our test results.

The objectives of our Quality Control system are:

- Use Bar-Coded operations to enable full traceability throughout the sample flow process and to ensure sample handling procedures and environmental conditions are managed well and there is no or minimal affect on the results.
- Continually improve the practices of our clients, franchise partners, associate doctors, clinics and hospitals and monitor their training needs. Be proactive in identifying gaps in the processes being followed. Guide them to ensure that the patients are served in the best possible way.
- Report the results with accuracy and clarity in a timely manner. Do a root cause analysis whenever there is a deviation against protocols and find solutions to the identified causes.
- Ensure a continual enhancement, implementation and maintenance of the quality system and seek improvement in the effectiveness of the quality system from experts at regular intervals.
- Meet and exceed expectations with respect to turn-around time, sample collection hygiene & reliability of service.
- Ensure that each test is performed by qualified and trained staff. Provide opportunities to the staff so that they can increase their knowledge and use the same for self and organizational betterment.
- Ensure that the equipment used are best in class, properly maintained and calibrated and where possible, measurements are traceable to recognized standards. Also explore methods which may lead to improvement in equipment performance and methodologies used for conducting tests.
- Enable technology upgrades to achieve higher accuracy and reduced complexities.
- Use internal audits and other checks to ensure the quality system complies with requirements; ensure problems are investigated promptly, root cause(s) established and effective action taken to prevent a recurrence.
- Have a smooth communication mechanism to ensure information is made available as rapidly as possible to those who need it, both internal and external to the organization.
- Monitor, help and support our franchise and service partners to be sensitive on all aspects of service delivery and to ensure quality standards are followed with no exceptions.

CONDITIONS of REPORTING

01. It is presumed that the specimen accompanying the TRF (Test Requisition Form where the details of patient are recorded) is of the same patient whose details are there in the TRF.
02. A test requested might not be performed due to the following reasons(s):
 - 2.1 Insufficient quantity of specimen required to conduct the test.
 - 2.2 Poor quality of the Specimen not meeting the quality criteria (hemolysis of sample/clotted.)
 - 2.3 Incorrect specimen type as required to conduct a test.
03. Test(s) may be partly or fully cancelled due to incorrect test code, incorrect name of the test or incorrect type of specimen. A communication shall be made and it is expected that a fresh specimen will be sent to laboratory for analysis of same parameter(s).
04. The results of laboratory investigation are dependent on the quality of the specimen as well as the assay procedures/technologies used. All samples collected for tests are required to be prepared, stored, labeled and brought to processing laboratory as per the prescribed guidelines of GENERAL DIAGNOSTICS.
05. GENERAL DIAGNOSTICS laboratory cannot be held liable for incorrect results of a sample which deviated from the guidelines issued.
06. There can be several factors like sample's unintended exposure to heat or travel through rough terrain which affect the quality of test results. Therefore a 2% chance of error/deviation in results is a possibility.
07. For certain category of tests, the report may carry a "PRELIMINARY" status implying that the results are yet to be reported for one (or more) tests. For example, in the case with certain microbiology tests, a "FINAL" culture, identification or drug susceptibility result might be pending. In such case, the status "RESULT PENDING" will be mentioned on report. The same shall be replaced by the test results whenever it is ready.
08. If the collection date or any other details was not stated in the Test Requisition Form, the same will not be printed on the report. In cases where the missing information is mandatory for report generation or meeting accreditation guidelines, the sample shall not be processed at all.
09. Tests parameters excluded from the "scope" of NABL accreditation shall be marked by asterisks.
10. In case you are not the intended recipient of the report, please immediately inform the same to the issuing entity. Any use, disclosure, copy or distribution of any contents of such report, is unlawful and is strictly prohibited.
11. Some test may be referred to other laboratories to provide a wider test menu to the patients. The details of the laboratory where the sample was referred to, can be obtained from Customer Care department.
12. Claims of comparing results against that from a different laboratory shall be looked into only if it was the same sample which was split and sent in same conditions to all laboratories and processed on the same technology.



The greatness of a NATION is judged by the way it treats its ANIMALS.